



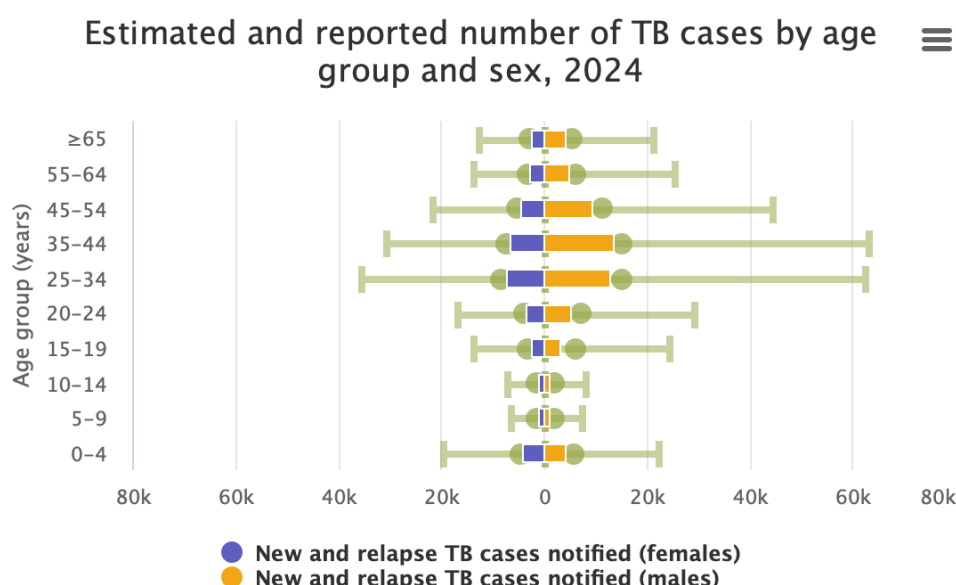
Advancing Age and Gender Responsive Tuberculosis Prevention and Care in Kenya: LIGHT Consortium impact

Tuberculosis in Kenya

Kenya is among the World Health Organization 30 high burden tuberculosis (TB) countries. While significant progress has been achieved with a reduction of 45% and 58% in incidence and mortality respectively in 2024 compared to the 2015 baseline, this is still below the END TB strategy targets. In 2024, Kenya notified a total of 97,246 persons with TB achieving a TB treatment coverage of 81% (WHO Global TB report, 2025).

TB Gender and Age

Men are disproportionately affected by TB accounting for 63% of the total persons notified with TB. (National TB data, 2024) Across all age groups 15 and above, males are consistently higher in TB notifications peaking in the 35 - 44 age group where males (14,023) nearly double females (6,712).



Source: [WHO Kenya TB Profile 2024](#)

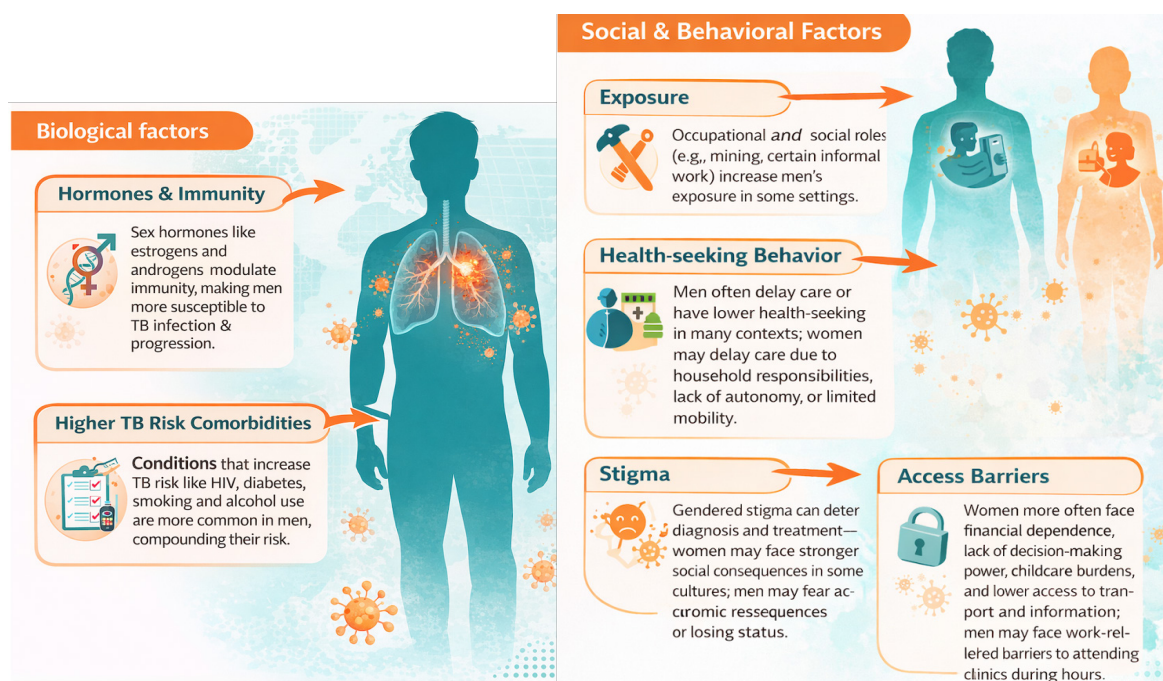
The 25 - 34 and 35 - 44 age groups account for the highest TB case burden, with a combined total of 41,232 (43%) cases. This is the economically active population, implying socio-economic impact due to lost productivity. The age distribution of notified TB cases in Kenya showed that children aged 0–14 years represented 13.3% (12,884 cases) of the total burden. Older adults (65+) show a notable burden at 7% (6,784).

Why the focus on TB in adolescents and young adults, and men?

Adolescents and young adults (AYAs) aged 15 to 24 years represent a significant proportion of the global TB burden. In Kenya, a review of the national TB surveillance data (2017-2022) revealed that this age group accounts for 17 % of all notified TB cases in Nairobi County and 15% nationwide. Despite these figures, critical gaps persist along the TB care cascade, resulting in missed opportunities for diagnosis, treatment initiation, and successful treatment completion.

They face unique barriers, including school and work commitments, stigma, and financial constraints, which affect retention in care. In addition, 20 percent of people lost to follow-up in Kenya are adolescents and young adults.

Why do men have higher rates of TB?



TB in men is driven not only by biology, but also by social and structural factors. These deepen gender inequities and limit access to timely, high-quality care. Men aged 15 years and over are especially affected and are often missed by TB services.

The LIGHT research programme, a six-year cross-disciplinary global health research programme, funded by UK aid, and led by Liverpool School of Tropical Medicine (LSTM) in collaboration with partners in Kenya, Malawi, Nigeria, Uganda, and the UK, **aimed to support policy and practice in transforming gendered pathways to health for people with TB in urban settings.**

What did the LIGHT research programme do in Kenya?

Respiratory Society of Kenya (ReSoK) and African Institute for Development Policy (AFIDEP) generated evidence on gender-transformative approaches to TB prevention and care, especially in high TB burden, urban contexts.

The research focused on understanding the burden of TB among AYAs — with particular attention to young men, who often have high TB prevalence but are less likely to complete screening, diagnosis and treatment, as well as patient-centred TB care.

The research studies included:

- a. TB care cascade analysis to identify where young people drop out of the care process.
- b. Qualitative research through interviews with young people who have and have not completed TB care to understand barriers, challenges and opportunities.
- c. Participatory research and co-design workshops involving young people and stakeholders (policy makers, health care workers) to develop tailored, patient-centred and gender-responsive interventions.

I. TB Care Cascade analysis

We analyzed data from the national TB management system (**TIBU**) and related tools for **2017–2022**.

Aim: Identify points where AYAs drop out along the TB care cascade, and to check for gender differences at each stage, including screening, diagnosis, treatment initiation, and outcomes.

Findings: Identified important gaps in TIBU, highlighting the need to strengthen age- and sex-disaggregated reporting, and predictive analytics to detect youth at risk of poor outcomes and enhance gender-responsive interventions.

II. TB among adolescents and young adults in Kenya

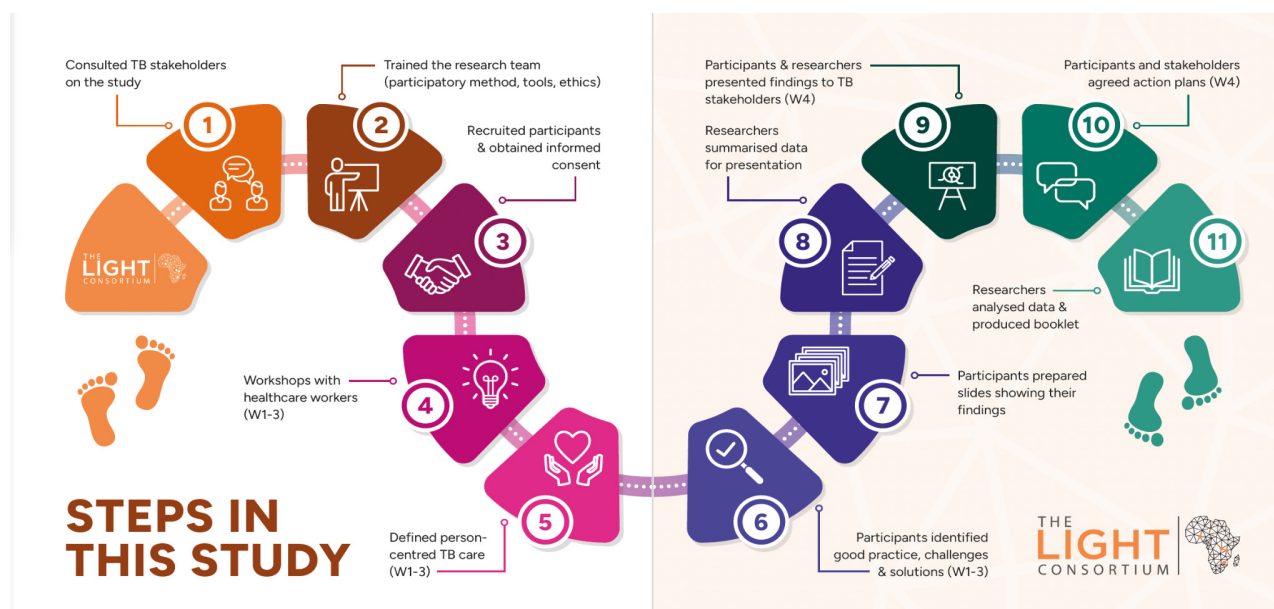
We analysed data from six high-volume TB sites in **Nairobi** to track young people's progress through the TB care cascade, identify drop-off points, and aid tracing for those disengaged from care. Transect walks pinpointed youth gathering spots near selected facilities, and site visits with youths and premises owners highlighted factors influencing TB transmission and treatment adherence. Healthcare workers recruited AYAs who completed treatment for workshops on improving youth- and gender-responsive services, while interviews with those who discontinued explored barriers. Findings were shared with stakeholders via structured forums to support practical action and local quality improvement.

Key barriers for AYAs in TB care include delayed diagnosis leading to severe illness, health system issues like long waits, inconvenient hours, inflexible appointments and drug refills, inconsistent confidentiality, and disruptions from worker strikes. **Social challenges** such as stigma, emotional distress, and homelessness also hinder access.

Treatment engagement enablers are supportive healthcare workers who follow up on missed appointments, provide clear communication, maintain a non-judgmental attitude, and tailor services to individual needs.

III. A participatory study with healthcare workers and policymakers on patient-centred care in Nairobi

We explored the policy-practice gap in patient-centred TB care in Kenya by engaging healthcare workers in three participatory workshops to share experiences, identify challenges, and propose solutions. A subsequent stakeholder workshop allowed participants to review these findings and collaboratively set goals and action steps for improving TB care.



Healthcare workers identified practices for differentiated TB care across age groups, genders, occupations, and comorbidities. However, implementation of patient-centred care faced multiple challenges including structural, health system, community and patient's individual constraints. Shortages of funding, workforce, and capacity affected close-to-community TB screening. Insufficient testing capacity at lower-level facilities led to diagnostic delays. Inflexible clinic hours and medication stockouts requiring frequent facility visits undermined TB treatment and adherence among working populations. Weak collaboration and referral systems limited support for people facing homelessness, substance use, or poverty.

https://light.lstmed.ac.uk/sites/default/files/content/centre_page/attachments/LIGHT%20PB%20PAR%20Nairobi_FINAL_16%20Nov%202024.pdf



What did we learn from LIGHT research in Kenya?

It is critical to disaggregate TB data by age and sex – helps to unearth population groups that are being left behind

It is important to understand what is driving gaps in the TB care cascade (the role of qualitative studies)

Adolescents and Young Adults are a critical population group that must be empowered to own their health and advocate for youth responsive care.

Tuberculosis services at facility level can be differentiated, patient-centred, age-and-gender responsive with a huge impact on TB notification.

TB stigma can be reduced and TB appropriate health seeking behavior enhanced using men centric approaches

Ending TB requires, engaging all service providers, addressing social issues and multisectoral engagement

Impact of LIGHT research and engagements in Kenya

AFIDEP and ReSoK engaged consistently and meaningfully with key stakeholders including the Division of TB and Lung Diseases, affected communities and healthcare workers, and other stakeholders to better understand the challenges and co-create solutions that inform national policies, sub-national practices, and improve age and gender-responsive TB services for all, with a particular focus on adolescents and young adults.

a) Highlighting and amplifying TB issues related to gender and youth in national discussions

The LIGHT Consortium has been instrumental in highlighting men bearing disproportionate share of the TB burden yet underserved due to harmful notions of masculinity and health systems that lack gender-responsive services. LIGHT has also brought the unique needs of adolescents and young adults (AYA) to the fore, advocating for youth-responsive services and disaggregated data to unearth and address the critical gaps this vulnerable group faces in the care cascade.

LIGHT has also empowered the youth with a voice to advocate for their TB rights and be part of national and global discourse on TB.

b) Contributed to shaping national policies on TB and gender



- Contributed to the **TB, Leprosy, and Lung Diseases National Strategic Plan 2023/24–2027/28**. The LIGHT team worked with stakeholders to co-create gender transformative strategies for improved TB care that address and overcome human rights and gender barriers, promoting equitable access to services for marginalized and vulnerable groups.
- LIGHT partners assisted in reviewing the draft “Policy on Tuberculosis Response in Learning Institutions, Kenya (2025–2035)” and provided evidence from their studies.
- LIGHT findings have been shared nationally via conferences, research forums, and media to raise awareness and maintain TB-and-gender as a priority for policymakers, partners, and frontline workers.

c) Generated evidence to inform practice

From Evidence to Action

Early Facility-Level Improvements in TB Services Following Research Findings

Background

This research focused on **adolescents and young adults (AYAs)** across 6 health facilities in Nairobi County. Following dissemination of findings, health care workers and community stakeholders identified practical improvements to make TB services more youth- and gender-responsive, patient-centred, and accessible.



Youth-Friendly TB Service Delivery

- Youth-focused TB clinic days
- Young clinicians and nurses supporting young people
- Use of youth-friendly language during consultations



Patient-Centred Treatment Support

- Flexible TB drug refills and alternative collection days
- Longer refills for working and school-going clients
- CHPs supporting DOT at homes/workplaces



Improved Communication & Engagement

- Youth-friendly communication approaches
- Use of WhatsApp and facility communication platforms



Strengthened Community Linkages

- CHPs sharing TB information and referrals
- Engagement of community chemists/pharmacies
- Referral forms from CHPs and chemists

Key Message

Research findings are already informing practical improvements in TB service delivery, demonstrating how locally generated evidence can strengthen youth-friendly, patient-centred TB care.

- Research findings are already informing practical improvements in TB service delivery. Following the dissemination of LIGHT research findings in Nairobi County, the health care workers and community stakeholders came up with practical improvements to make TB services more patient-centred, youth and gender-responsive.
- The County Health management has also made commitments to scale up TB differentiated care models, integrate school-based TB services integrated in ongoing activities, Youth-Centred Communication, and age- and gender responsive care.
- Sex and data disaggregation along the TB care cascade- The National TB program noted that t-bu Lite roll out will enable disaggregated data all along the care cascade including screening, which was noted to be a gap during the research.
- Although not from our study, NTP has implemented predictive analysis in TIBU to enhance differentiated care, identify those at risk of poor outcomes, and provide age- and gender-responsive interventions.

d) Contributed to capacity strengthening and improved research skills in Kenya

- **Evidence-Informed Decision-Making** trainings strengthened capacity of decision-makers, researchers and implementers to understand, communicate and use evidence. Our workshops and training sessions focused on applying evidence in programmatic decisions, not only generating findings but improving ability to use them.

- Supported the Kenya TB Research Task Force

In early 2025, the TB program initiated the establishment of a research taskforce and the development of a research framework to advance scientific knowledge and innovation in TB and lung health. ReSoK and AFIDEP, became **core members of the secretariat contributing** to the formation of the taskforce and development its terms of reference. LIGHT consortium offered **technical, logistical and funding support** for the first biannual research dissemination forum held in May 2025.

Kenya's Research Taskforce now functions as a national platform that supports **shaping the country's TB research agenda** fostering ongoing collaboration and structured engagements between researchers and policymakers. This ensures that evidence generated by research organizations and consortia such as LIGHT, is shared and discussed, thereby informing decision-making processes like the National Strategic Plans, program reviews, M&E tool revisions, service delivery improvements among others.

- Scientific Writing Workshop to advance TB Research Capacity

Responding to identified needs, the task Force facilitated a **five-day Scientific Manuscript Writing Workshop** in July 2025 (40+ participants across institutions, ~16 manuscripts drafted). LIGHT co-funded and supported facilitation, ensuring outputs are tracked either towards publications or final drafts of manuscripts. The workshop improved research writing and publication skills for TB and lung health researchers, leading to 11 manuscripts submitted by March 2026.

- Training on understanding Concepts and Research Methods in Kenya

The research methods training equipped participants with practical skills to use evidence in health policy, particularly for national TB programs. It covered health economics, quantitative and qualitative approaches, TB modelling, economic evaluation, data analysis, simulation tools, and participatory strategies for inclusive policymaking. Sessions stressed combining methods and considering gender and equity for evidence-based decisions. The Kenya TB program formed a multidisciplinary Modelling and Analytics Coordination working group to base decisions and resource allocations on scientific evidence.

e) Enhancing media reporting on TB and Gender

Health journalists are essential in shaping public understanding of diseases. The LIGHT Consortium's collaboration with TB program, and development partners to train journalists resulted in stronger reporting on TB and gender-related topics. This led to greater media coverage across radio, TV, print, and social media, amplifying lesser-known TB stories, addressing stigma, and highlighting how gender impacts TB care and access. Media interest in TB from a gender perspective also increased.

f) Engagement of parliamentarians

LIGHT collaborated with the Network of African Parliamentary Committees of Health (NEAPACOH) to highlight how gender affects TB risk, diagnosis, and outcomes—especially the greater burden on men due to delayed care-seeking and poorer results compared to women and children. These efforts have further driven recognition of TB as a national and economic priority, underscoring the need for more domestic resources, attention to social determinants, government accountability for END TB targets, and the integration of gender-responsive policies into national TB strategies.

Key recommendations

Monitoring and evaluation systems to support TB age- and Gender programming

Institutionalize routine sex- and age-disaggregated data collection, analysis, and reporting across the entire TB prevention and care cascade. Introduce gender-specific TB indicators to enable systematic monitoring of gender disparities in TB services, and during program reviews. These should include community-level indicators and data on key and vulnerable populations, to guide the design and implementation of targeted, evidence-based interventions. Support the utilisation of T-bu lite platform by all healthcare workers.

Scale up male focused interventions

Meet men where they are by expanding screening into male-dominated social and economic hubs such as construction sites, transport hubs, and sports venues. Community-based case finding, workplace-integrated care and innovations such as self screening, and use of Xray should be scaled up. These efforts should be supported by health system reforms that address structural barriers, including flexible clinic hours and the recruitment of more male healthcare workers. Addressing harmful gender norms that frame health-seeking as weakness is also essential to improve men's engagement and reduce transmission to families and communities.

Expand differentiated and youth-responsive services

Programs should provide welcoming clinics with flexible hours and faster service to accommodate school and work schedules. Ensuring confidentiality through private consultations and discreet packaging is essential, as is training healthcare workers in non-judgmental, gender-responsive communication. By decentralizing services to schools and workplaces, integrating peer support networks, and utilizing SMS follow-ups, providers can reduce stigma and improve care coordination. Ultimately, these operational shifts must be backed by youth-centred policies that use evidence from adolescents and young adults to fund and implement targeted, gender-responsive national strategies.

Decentralize and Integrate TB services into PHC

Health systems should accelerate decentralizing TB services and integrate them into Primary Health Care (PHC). Adopting near point-of-care diagnostics and community treatment support, programs can reduce catastrophic costs like travel and lost wages that affect patient retention.

Integration should follow a “one-stop-shop” approach, systematically embedding TB screening into HIV care, maternal health, diabetes management, and support for undernutrition and substance use. Strengthening healthcare worker skills and involving all providers—including private and community pharmacists/chemists—will help ensure prompt identification and referral of symptomatic individuals to quality care, regardless of their initial point of contact.

Tailor TB information and platforms to intended audiences

To maximize program reach, TB communication should align with the social and digital habits of men and youth to address stigma and gender barriers. Targeted messages in male-focused spaces like transport terminals, entertainment and sports venues, barazas, and relatable content on TikTok and Instagram for adolescents, are recommended. Peer-led advocacy and campaigns should challenge stigmas around seeking care, promoting early diagnosis and improved treatment adherence.

Financing of youth and gender responsive TB interventions

Youth and gender-responsive interventions require financial support. The TB program needs to include to prioritize these in budgets, Global Fund proposals, US government mechanisms, and other funding requests. Increased domestic funding for TB and gender-related programs should be advocated.